Product X: Provider Net Cost Model Concept Brief

Provider economics and access execution brief

Audience: Provider-facing access stakeholders and internal MLR reviewers

Prepared March 30, 2026. Facts reflect the attached source set and public CMS, HRSA, Medicaid, Medicare, and payer policy sources reviewed on March 30, 2026.

Why the provider model matters

Expanded access experience is not a commercial reimbursement proxy. Under FDA expanded access, manufacturer willingness to supply the investigational drug is the gatekeeper. In commercial launch, the gatekeeper becomes a payer-mediated system of prior authorization, coding, sourcing rules, site-of-care controls, claim edits, and reimbursement timing.

A provider net cost model translates that shift into auditable economics. The model measures whether a treating center recovers drug acquisition cost and cash exposure after reimbursement mechanics are applied. It does not rely on a generic allowed amount narrative. It uses the actual launch variables that a center will face: acquisition price, whether the plan permits buy-and-bill, whether a 340B price is available, and whether Medicaid claim rules are met.

The source brief anchors the launch arithmetic at a WAC of $39,000 per 900 mg vial administered every two weeks. That one figure is enough to make provider exposure concrete. At 26 administrations per year, gross annual drug WAC is $1,014,000 before administration costs or denied claims are counted.

Commercial payer perspective and treatment of similar products

Comparator policy	Initial gate	Continuation gate	Economic signal for adrabetadex
UnitedHealthcare commercial - Miplyffa	Genetic confirmation, use with miglustat, no Aqneursa, specialist involvement, 12-month authorization	Positive clinical response documented on NPC-specific scales	Coverage exists, but combination use and ongoing response are managed tightly.

Comparator policy	Initial gate	Continuation gate	Economic signal for adrabetadex
UnitedHealthcare commercial - Aqneursa	Genetic confirmation, neurologic manifestations, with miglustat or documented inability to use miglustat, no Miplyffa, 12-month authorization	Positive clinical response documented on NPC-specific scales	The commercial market already treats NPC therapy as a managed category, not an open category.
Louisiana Blue - Miplyffa	Age and weight gate, genetic confirmation, ambulatory status, no adult-onset disease, with miglustat, no Aqneursa	Improvement or stabilization in ambulation, fine motor function, swallowing, or speech	Some plans will use additional functional restrictions beyond the FDA label.
Aetna - Spinraza / Brineura	Baseline assessments plus specialist administration requirements	Positive response or slowed loss of function from baseline	Intrathecal and intraventricular analogs already tie payment to measurable continuation rules.

The commercial payer takeaway is direct: product X will launch into a market where high-acuity pediatric neurologic drugs are covered, but they are covered with explicit utilization management and measurable continuation standards.

340B pricing explained in plain language

340B changes provider acquisition cost, not payer coverage criteria. HRSA's core rule is simple: a manufacturer may not charge a covered entity more than the 340B ceiling price, whether the entity buys through a wholesaler or directly from the manufacturer. The ceiling price equals Average Manufacturer Price minus the Unit Rebate Amount.

The Unit Rebate Amount uses minimum rebate percentages that differ by product type. The current 340B / Medicaid framework uses 23.1% for most brand-name drugs, 17.1% for qualifying brand-name pediatric drugs and clotting factor products, and 13.0% for generics. For a new drug, before AMP is available, HRSA requires manufacturers to estimate the 340B ceiling price using WAC minus the applicable rebate percentage.

Using the launch WAC in the source brief, a 23.1% new-drug estimate produces a 340B acquisition price of $29,991 per 900 mg vial. If adrabetadex is ultimately treated as a qualifying pediatric brand drug for this purpose, the same calculation at 17.1% would imply $32,331 per 900 mg vial. That means the modeled 340B discount range is $6,669 to $9,009 below WAC on every administration, depending on final program classification. For centers that qualify for 340B, this is a material provider-economics lever. For the manufacturer, it is a material gross-to-net drag.

340B also creates compliance work. HRSA prohibits duplicate discounts, meaning the same unit cannot generate both a 340B discount and a Medicaid rebate. Covered entities that carve in Medicaid fee-for-service claims must keep the Medicaid Exclusion File listing accurate and must align carve-in or carve-out status with actual billing practice.

How the product behaves in Medicaid and for dual-eligible patients

Medicaid is the most important access engine in the modeled launch. The internal payer mix is 50% Medicaid. EPSDT stands for Early and Periodic Screening, Diagnostic, and Treatment. Medicaid.gov states that EPSDT requires states to cover medically necessary services needed to correct or ameliorate physical or mental conditions in beneficiaries younger than 21, including Medicaid-coverable services that are not otherwise listed in the state plan. In practice, that makes pediatric medical necessity the strongest coverage tailwind for adrabetadex.

EPSDT is a coverage rule, not a single national reimbursement formula. Physician-administered drug payment remains state-set. Current official state-plan examples show both WAC-based and ASP-based reimbursement. Colorado reimburses physician-administered drugs in the Medicare ASP Drug Pricing File at ASP minus 3.3% and reimburses drugs outside that file at WAC. Pennsylvania pays brand prescriber-administered drugs at the lower of the provider's usual and customary charge or WAC plus 3.2%.

EPSDT does not remove claim mechanics. Medicaid's Physician Administered Drugs program ties payment availability and federal matching funds to state collection and submission of utilization data and, for defined categories, NDC coding. A clinician-administered adrabetadex claim that fails NDC capture or other PAD edits will not convert cleanly to cash.

Dual-eligible volume is de minimis in the initial infantile-onset launch population because the labeled population is pediatric. When a dual-eligible beneficiary does receive a Medicare-covered clinician-administered service, Medicare pays first and Medicaid pays last. Medicaid therefore functions as secondary wraparound coverage, not as the primary drug payer, for Medicare-covered services.

One operational point is critical in 340B settings: HRSA's Medicaid Exclusion File applies to Medicaid fee-for-service, not to Medicaid managed care organizations. The model therefore separates Medicaid fee-for-service, Medicaid managed care, 340B carve-in, and 340B carve-out scenarios instead of averaging them together.

HCPCS and pass-through status: impact, timing, and required data

HCPCS coding and OPPS pass-through status determine whether hospital outpatient claims are visible, payable, and administratively manageable. CMS states that pass-through status is a temporary additional outpatient payment for qualifying new drugs and biologicals while the agency gathers cost data. CMS also states that pass-through runs for at least 2 years but not more than 3 years. For providers, the impact is practical: pass-through can reduce packaging pressure in hospital outpatient departments during early launch, while the absence of product-specific coding increases use of miscellaneous billing, manual review, and denial risk.

Pass-through status does not replace payer management. It affects hospital outpatient Medicare payment mechanics. It does not eliminate prior authorization, commercial medical-benefit controls, white-bagging rules, or site-of-care edits. For adrabetadex, the operational value is that a clean HCPCS and pass-through path can make hospital outpatient billing more predictable during the launch period while claims data accumulate.

CMS now requires OPPS pass-through applications through MEARIS. Drug and biological applications are reviewed quarterly. A complete file received by the first business day of March, June, September, or December can take effect on July 1, October 1, January 1, or April 1, respectively. Using the current August 17, 2026 PDUFA date, the earliest practical pass-through effective date would be January 1, 2027 if the approval letter, final label, pricing file, and utilization package are complete in time for the first business day of September 2026.

- The HCPCS and pass-through package will need the FDA approval letter, final label, trade and generic name, proposed descriptor, dosage form, package size, method of administration, and billing unit logic.
- CMS states that the application must also include WAC and AWP, ASP if available, actual hospital cost, market-availability date, projected utilization by site of care, and the requested HCPCS code or descriptor.
- If product-specific outpatient coding is not active at launch, hospital outpatient claims may need the interim miscellaneous-code bridge such as C9399 or NOC billing under MAC rules, which is exactly why the coding file and billing guide must be ready before the first broad claims wave.

Small provider demo model using public benchmarks and launch inputs

The table below isolates drug spread only. It excludes administration revenue, anesthesia, imaging, and staffing cost because those amounts vary by site and payer. The purpose is to show what happens to provider economics before procedure revenue is layered in.

Scenario	Acquisition cost per dose	15% ref / dose	20% ref / dose	30% ref / dose	Drug payment benchmark per dose	Gross drug spread per dose	Annualized spread at 26 doses	Interpretation
Commercial buy-and-bill at 100% of WAC	$39,000	$33,150	$31,200	$27,300	$39,000	$0	$0	Provider economics depend entirely on procedure revenue, denial performance, and payment speed.
Medicare Part B initial sales benchmark at 103% of WAC	$39,000	$33,150	$31,200	$27,300	$40,170	$1,170	$30,420	Public benchmark spread is thin before administration cost and rework cost are applied.

Scenario	Acquisition cost per dose	15% ref / dose	20% ref / dose	30% ref / dose	Drug payment benchmark per dose	Gross drug spread per dose	Annualized spread at 26 doses	Interpretation
340B covered entity using the same 103% benchmark	$29,991	$33,150	$31,200	$27,300	$40,170	$10,179	$264,654	340B changes the provider economics immediately and materially.
Commercial white-bagging	$0	$33,150	$31,200	$27,300	$0	$0	$0	The specialty pharmacy bills the plan directly, and the provider loses drug spread while retaining operational work.

Benchmark math: the 15% reference price is $39,000 x 0.85 = $33,150 per dose, the 20% reference price is $39,000 x 0.80 = $31,200, and the 30% reference price is $39,000 x 0.70 = $27,300. The Medicare initial-sales benchmark uses 103% of WAC, or $40,170. The new-drug 340B estimate uses $39,000 x (1 - 0.231) = $29,991.

Operating-margin sensitivity: procedure revenue, denial performance, payment speed, and labor time

Provider economics do not stop at drug spread. A live provider model also needs procedure revenue, clinician time cost, denial performance, and payment speed because those items determine whether cash actually turns into margin. The table below adds those operating drivers to the non-340B commercial buy-and-bill scenario.

These are demo-model inputs, not fixed national rates. The illustration uses $750 of procedure revenue per administration, 1.25 hours of physician time costed at the BLS May 2023 pediatrician mean hourly wage of $98.97, and 3.0 hours of nurse time costed at the BLS December 2024 registered-nurse wages-and-salaries rate of $50.21. Denial drag is modeled at 0.5% of total billed revenue, and payment-speed cost is modeled as 45 days to payment with an 8% annual carrying-cost proxy on the $39,000 drug acquisition.

Scenario	Procedure rev / dose	Physician + nurse time / dose	Denial drag / dose	Payment-speed cost / dose	Mfr support / dose	Net op margin / dose	Annualized at 26 doses
Commercial buy-and-bill, no extra support	$750	$275	$199	$385	$0	-$109	-$2,834

Scenario	Procedure rev / dose	Physician + nurse time / dose	Denial drag / dose	Payment-speed cost / dose	Mfr support / dose	Net op margin / dose	Annualized at 26 doses
Commercial buy-and-bill + 5.0% manufacturer discount or equivalent rebate	$750	$275	$199	$385	+$1,950	$1,841	$47,866
Commercial buy-and-bill + 10.0% manufacturer discount or equivalent rebate	$750	$275	$199	$385	+$3,900	$3,791	$98,566

Illustrative labor math: 1.25 x $98.97 = about $124 of physician time and 3.0 x $50.21 = about $151 of nurse time, for about $275 total. Illustrative manufacturer-support math: 5% of a $39,000 WAC equals $1,950 per dose and 10% equals $3,900 per dose.

What the provider model will include

- A commercial buy-and-bill scenario, a commercial white-bagging scenario, a Medicaid fee-for-service scenario, a Medicaid managed care scenario, and separate 340B carve-in and carve-out scenarios.
- Drug acquisition cost, drug payment benchmark, claim-denial sensitivity, days-to-payment, and claim-cleanliness inputs for NDC capture and documentation completeness.
- A distinct output for provider cash exposure because a center that buys product at WAC every two weeks carries more risk than a center that only bills administration.
- A separate assumption set for hospital outpatient departments and physician offices because site-of-care policies already differentiate those settings.

Bottom line

The provider model does not prove broad coverage. It proves where provider economics break, where they remain neutral, and where they become favorable. On the supplied inputs, non-340B buy-and-bill economics are thin, 340B economics are strong, and white-bagging removes drug spread entirely.

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